

Addiction Is Not Adaptive Preference: A Feminist View on A Psychiatric Puzzle

Levi Lowe

Department of Philosophy, University of Utah

Statements and Declarations

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Correspondence concerning this article should be addressed to Levi Lowe, University of Utah, 201 Presidents' Cir, Salt Lake City, UT 84112, United States.

Email: levi.lowe@utah.edu

ORCID iD: 0009-0001-0095-9399

1. Introduction

Imagine a person and call him Marty. Marty claims to successfully depend on alcohol. That is, he has a goal, uses alcohol to successfully achieve it, and accepts all the consequences involved. The success of the proclaimed dependence is rated purely teleologically. That is, the current model of alcohol use disorder (AUD) says that Marty does not have AUD because the alcohol-dependency here is teleologically successful. In this paper, I ask: does that understanding of addiction make sense? I answer: yes.

The best case of an apparently successful dependence, yet one that we still might want to call an addiction, is what feminist philosophical literature has termed “adaptive preference.” Briefly, an adaptive preference is a preference that was developed in response to deprived circumstances. We might want to say that Marty has an adaptive preference for alcohol, and it thus counts as an addiction. As philosopher Rosa Terlazzo (2018) writes, “...it might be the case that before the adaptive preference is formed, the agent would be made prudentially worse off by forming the adaptive preference than by forming a nonadaptive alternative.” Meaning: even though Marty is claiming his relationship with alcohol is successful *now*, that relationship might have been unsuccessful *during its onset*. If something used to be imprudent, but is now prudent, and these temporally separated competing realities exist, then can we call it a case of addiction? No. We cannot, or so I will argue. If a non-deceived individual claims to have a successful dependence, then it is inaccurate to diagnose her with addiction, even if the successful dependence counts as an adaptive preference because adaptive preferences are successful dependencies.

Roadmap

What follows is a *reductio ad absurdum*. That is, I will charitably showcase the best argument that adaptive preferences *do* count as addictions, and demonstrate why making such a commitment leads to absurd conclusions. To be clear, however, my feminist interlocutors such as Terlazzo and Khader are not really my opponents here, despite my target, because their accounts do not claim to inform addiction diagnosis. My goal here is to preempt reasonable rejections to the current model of addiction, and I think it is reasonable to suggest that the satisfaction of an adaptive preference counts as an addiction. However, this suggestion ultimately does not make sense and deserves a focused explanation.

First, I will reintroduce the prime example at hand, Marty, and discuss why he is a puzzle for the current account of addiction. I will then introduce two strong solutions to the puzzle, namely, two versions of “adaptive preference.” I will opt for one of these accounts—namely, philosopher Serene Khader’s (2011) deliberative perfectionist approach to adaptive preference. Khader says that people can have preferences, ones that they endorse in deprived circumstances, but that are ultimately inappropriately adaptive when the preferences undermine one’s flourishing. That is, if the preference is one that an agent would not endorse in flourishing circumstances, it is an inappropriately adaptive preference. I will show how one would use it as a lens through which the puzzle is solved, a lens through which we would call Marty a case of adaptive preference satisfaction and thus addiction. Ultimately, however, solving the puzzle this way will end up committing oneself to a psychologically absurd conclusion: that one’s actions should be motivated by hypothetical counterfactuals concerning the context in which one’s preferences could have ideally developed. My aim will be to show that, while adaptive preferences shed light on the contexts in which preferences develop, thereby providing moral relief from strange or apparently deleterious contextually learned preferences, the notion of an

“adaptive preference” construed this way does not claim that Marty’s actual relationship with alcohol is pathological. Nothing has gone wrong psychologically with respect to how goals are achieved. Thus, while the satisfaction of an adaptive preference undermines one’s counterfactual sense of flourishing, it does not undermine their current psychological capacity to rationally achieve goals.

We need to agree on two things before I begin my argument: basic assumptions about the psychiatric institution generally. Firstly, if a doctor diagnoses a patient with a mental disorder, the doctor is at least claiming that this person would benefit from treatment. That claim comes from one of the main bioethical principles: beneficence. Secondly, psychiatric diagnoses are about helping, specifically, psychological issues—or “mental disorders.” We do not treat broken limbs or diabetes by sending them to psychiatrists. So, if a psychiatrist diagnoses a patient with an addiction, but the patient’s issue is not psychological or otherwise mental, then it is a misdiagnosis. These are our foundational premises that will not be defended in this paper.

2. The Marty Puzzle

Consider the following story. Marty is deprived—shelter-deprived, nutrient-deprived, and deprived of a healthy upbringing. Not even the loosest conception of human-flourishing would deem Marty’s circumstances “flourishing.” Marty’s closest friend, Jennifer, decides to try helping Marty, but she is uncertain which area of Marty’s life should receive attention first. Readily witness to Marty’s persistent drinking, Jennifer decides to ask Marty about his relationship with alcohol.

JENNIFER: Marty, do you really want to be drinking so much?

MARTY: Yes and no. I want to feel better. Drinking makes me feel immediately better.

JENNIFER: But if you were in flourishing circumstances, rather than deprived ones, then you would drink less?

MARTY: Probably!

That is the story of Marty. Should Jennifer remain pursuing alcoholism treatment options for Marty? Among Marty's clear issues, is Marty pathologically using alcohol? I will argue that there is enough information to suggest that Marty is not addicted to alcohol. Jennifer should stop thinking of Marty as a pathological user of alcohol.

Marty's situation is a prototype. His "Yes and no" response echoes Terlazzo's (2018) argument that there is this dual reality separated by temporal framing whereby the adaptive preference is both prudentially good and prudentially bad. The general theme that his case represents is one in which a person's preference is entirely contingent on the deprivation he has grown accustomed to, such that switching to flourishing circumstances would eliminate the preference in question. We want to know if an individual like this should be classified as an addict. The reason this problem is so important is because, should it be a case of addiction, the current model of addiction would be facing an anomaly. A clinical case of pathological use makes itself known by being actively unsuccessful. Its signs and symptoms involve active distress on behalf of the patient. A person is actively upset by their usage. However, Marty does not appear unsuccessfully dependent on alcohol because he is only retrospectively upset. He is dependent on alcohol to achieve a particular end and he achieves said end without unacceptable consequences. So Marty appears successfully dependent. Thus, if it turns out that Marty is actually an addict, then these forms of addiction would be foundational refutations of the current lens. Fortunately for this lens though, I do not think cases like Marty's represent cases of addiction.

The philosophical literature that pertains most to our case of Marty is the feminist philosophical literature on “adaptive preference.” In other words, Marty’s preference for alcohol might be considered an adaptive preference. Adaptive preferences, as I will explain, are categorically different from run-of-the-mill preferences. As witnesses, we are urged to respond differently to them. As a run-of-the-mill preference, Marty’s relationship with alcohol would canonically not be an addiction. Understood as an adaptive preference, does that conclusion change? I will argue: no.

According to the current model of addiction, Marty is not addicted to alcohol because he is achieving his goals. But what if his stated preferences are not his *true* goals? Then he might be undermining his *true* goals by achieving his *less true* stated preferences, right? If he is persistently undermining his true goals, it appears that the current model of addiction should diagnose Marty with AUD.

If we can somehow discredit Marty’s testimony, we may be able to argue from there that his preferences are undermining his genuine goals. Broadly speaking, there are a couple of ways we can try to discredit Marty’s immediately stated preferences. On the one hand, we can say that Marty is self-deceived in some capacity. In cases like these, the current model urges us to use something like philosopher Natalia Washington’s (2018) contextualist approach to anti-subjectivist paternalism, and say that the patient is suffering from some distorted evaluation that is leading to their unfulfilled true goals (Sripada 2022). On the other hand, we can imagine the different versions of an individual across time, and pit them against each other. Consider the following approaches.

We might consider a dialogical approach. There is a chance that if we asked the current version of Marty whether his life fits his conception of a good life, he may sometimes report in the negative. The point of this paper, though, is that sometimes he reports in the positive. That is, sometimes, the costs of usage appear worth it from his perspective. So, perhaps if Marty's evaluation of his life were dialogical, such that a view of it across time would lead him to conclude that these preference-oscillations undermine his goals, then he would be an addict under the current model. This situation is possible, but not all-encompassing. Indeed, if we are ignoring the possibility of Marty's Sripada (2018) style of self-deception, we need to address the situations wherein Marty's dialogical goals are not being compromised. Again, I am here interested in questioning non-self-deceived preferences.

Some may have a hunch that Marty's new set of values are somehow not authentic, even if they are dialogically satisfactory. The thought goes: *How could someone ever genuinely be okay with losing their loved ones, jobs, and home? There is just something not right about that, no matter how "sure" Marty is.* There is then perhaps a second way we might try to discredit Marty's immediate preference-satisfaction. According to the feminist philosophical literature, one can develop preferences that coordinate with her circumstances, or "adaptive preferences." This delivers us a retrospective approach. It is plausible that if we asked a younger version of Marty about his current relationship with alcohol, he would report that the cost is unacceptable, that his goals are being undermined. He might report wanting to change his behavior around alcohol and drugs. This is what Terlazzo (2018) meant by "before the adaptive preference is formed, the agent would be made prudentially worse off by forming the adaptive preference." So, maybe some version of the current model that relies on one's younger conceptions of a good life could diagnose Marty with AUD. However, I am not satisfied with this method because it

seems to take the person's temporally relevant interests out of the conversation, forcing people to never change their goals. However, there are many understandings of adaptive preferences, and what we may find is that adaptive preferences are not as authentic as ordinary preferences, which may lead us to conclude that Marty is undermining his true preferences.

3. So, an Addict and a Feminist Walk into a Bar...

The term "adaptive preference" gets used in different ways within the literature, but the baseline theme is something like the following: an adaptive preference is a preference that is not freely selected.

The term 'adaptive preference' originally comes from Jon Elster, who used it to describe the phenomenon of sour grapes: the fox in Aesop's fable wants the grapes hanging above him, but upon discovering that they are out of reach, decides that they must have been sour and that he therefore doesn't want them anyway (Elster 1983). The example shows us the problem for social welfare calculations clearly. If a person's desire not to have a thing is a reaction to the fact that she *cannot* have it, then her preference not to have it seems to do a poor job of capturing her welfare.

Of course, preferences are hardly selected; we do not choose our tastes. But there is a sense in which a "free choice" is meaningful, and meaningfully absent in cases of adaptive preferences. Selecting among a three options is meaningfully different from selection among hundreds. Despite the vagueness of that description, hopefully it is starting to become clear why Marty might be thought of as having an adaptive preference. He lost his loved ones and jobs, and

developed preferences acclimated to those losses. He developed a deleterious drinking habit, and grew accustomed to this condition. His preferences align with his condition, but the preferences seem to have been more forced upon him than if someone else had the same preferences after a more fortunate upbringing. It seems reasonable, even, to say that the preferences are not attributable to him in the same way we would attribute responsibility to someone who is flourishing. If that is true, what does that mean for Marty's potential psychiatric conditions, or potential medical treatments? I will ultimately argue that it means he is not using alcohol pathologically. If we want to help Marty's well-being, we should not blame everything on his drinking habit.

The Non-Autonomy Model

Some feminist philosophers might argue that Marty's new set of values are not autonomously attributable to him. The sense in which Marty's new values are being called *non-autonomous* here is supposed to capture the sense in which the values are not his. Call this the Non-Autonomy model of adaptive preference. The classic example in this literature is the case of the "deferential wife" —a woman whose preferences she defers to her husband. The argument is that her preferences are not autonomous on account of a lack of a deliberative process. She doesn't have reasons of her own—that is, non-deferential reasons— for being deferential. She hasn't supported the reasons for her deference through a self evaluative deliberation. As Philosopher Andrea Westlund (2003) explains,

I take it that the reasons in question must be ones the person herself has canvassed and assessed, not ones she merely borrows from another and mouths by rote. But if, as I've

suggested, [the deferential wife's] deference is deep in the sense that it has no non-deferential grounds, how could she possibly endorse it for reasons she herself has canvassed and assessed? (Westlund 2003).

In other words, this view builds a substantive deliberative process into the definition of autonomy, such that a lack of this kind of deliberation would count as a lack of autonomy. That lack of autonomy signifies an adaptive preference and justifies coercive intervention. That is without an autonomy to disrespect, the blockade to paternalism collapses. We can imagine a similar 'deferential drinker.' She doesn't have deliberated reasons, or hypothetical defenses of her reasons, for drinking. Then the lack of autonomy would justify some sort of remedial intervention.

There are at least four reasons one should be averse to the Non-Autonomy model of adaptive preference, especially when used as a lens for addiction diagnosis. Firstly, and what should immediately divert our attention from this example, a lack of a deliberative process is not what makes Marty's case puzzling. Even if he defended his reasons for drinking—saying something like, "It relaxes me"—we are interested here in questioning the authenticity of these thick preferences. Secondly, in case one is still curious about a case like the deferential drinker, feminist philosopher Serene Khader finds it practically implausible that Marty is so unreflective about his motivational biases that we could call him non-autonomous. Thirdly, there are moral reasons against the Non-Autonomy model—namely, it would justify a significant amount of coercion. Merely locating a lack of what seems to be substantive deliberation justifies full-blown paternalism, under the Non-Autonomy model. I agree with Khader—the Non-Autonomy model allows for too much paternalism. It is not that we need a model which eliminates it altogether, but we need a model whose justification of paternalism is minimal. We need a model that saves

the important investigation of one's deliberative process, but tries to lessen the amount of justifiable paternalism and coercion. Fortunately, we will find these features in the next section, with Khader's deliberative perfectionist approach.

The fourth, and final, reason that we should avoid the Non-Autonomy model of adaptive preference is that it does little to inform the current model of addiction. The orthodox view of addiction—the 'irresistibility model'—would rather seem to predict the Non-Autonomy model of adaptive preference. That is, the irresistibility model of addiction characterizes addiction as a loss of autonomy to begin with. So the conclusion that a substance changing one's preferences also counts as a loss of autonomy corroborates the irresistibility model. However, the Non-Autonomy model is no help for the current model of addiction. All we would know from the Non-Autonomy model is that the goals Marty is achieving are not **his** goals. It does not follow that these achieved goals are discordant with Marty's well-being and psychological processes. Remember that we need the preference satisfaction to represent something having 'gone wrong' psychologically if we are going to treat Marty as a psychiatric patient. Marty could have an adaptive preference to love apples (perhaps because they were the only fruit available to him growing up). But that would not need addiction treatment. Or, as Terlazzo (2018) points out,

Imagine that as a teenager, I would have formed a preference for regularly using heroin if I had known how to get some. Because I did not know that, however, I decided to apply myself in school. On our definition, this is clearly an adaptive preference: studying hard was within my limited option set, and I would not have preferred it if my set had also included heroin use. But presumably anyone will grant that my adaptive

preference is in this case prudentially far better for me than the alternative. (Terlazzo 2018)

Thus, we need something more than a breach of autonomy to count as an adaptive preference that needs addiction treatment.

The Deliberative Perfectionist Approach to Addiction Diagnosis

Serene Khader (2011) might cache out the inauthenticity of Marty's new values through what she calls a "deliberative perfectionist approach"—whereby a vague baseline of human flourishing is used as a standard against which the content of stated preferences are compared. Under this model, we could call Marty's preferences inappropriately adaptive if he would reject them under conditions conducive to his flourishing, but accept them in deprived circumstances. He may 'be okay with it' while in deprived circumstances—as in, "I've learned to love it." But if he would have rather not needed to make this preference adjustment—if it is not something he would endorse in flourishing circumstances—then it is an inappropriately adaptive preference according to Khader. Khader's (2011) approach cleanly incorporates Terlazzo's (2018) definition: "A favoring of an option, O, developed in response to a constrained option set that includes O as a member." So Marty favors drinking and developed his preference in response to deprived circumstances in which drinking was an option for him. If he were not deprived, we would not endorse it.

However, Terlazzo (2018) clearly argues against Khader's (2011) view that adaptive preferences *must* frustrate one's interests. TeSo, for those trying to make the strongest argument

that adaptive preferences count as addictions, which are indeed necessarily conflicting with one's interests, it would be uncharitable to rely on Terlazzo's (2018) account over Khader's (2011) less agnostic stance.

Firstly, Khader's model does not rely on stated preferences in the same way that the Non-Autonomy model or Terlazzo's (2018) account relies on stated preferences. Khader's model relies on a vague and objective conception of human flourishing. One's stated preferences are compared to this vague conception of flourishing. One of the issues with the Non-Autonomy model is that autonomy is all it takes to assert that one's preference is authentic. For example, a Marty with autonomously defended preferences would not be an addict under the non-autonomy model. Khader's approach gives us an alternative solution. Under Khader's model, it does not matter how subjectively defensible one's reasons are; if the stated preference does not fit into at least the vague and objective conception of human flourishing, it is an adaptive preference.

Secondly, remember that our goal was to save the investigation into one's deliberative process while removing it from the need to invoke the notion of autonomy when discussing how one's immediate preferences bear on her well-being. By not invoking the notion of autonomy, having an adaptive preference does not mean that one's autonomy—which is the blockade to paternalism—disappears. So we get to minimize the justification for coercive intervention.

The charitable model here is Khader's approach. I will show how, with one amendment to the approach, the deliberative perfectionist approach seems useful as a lens through which we can identify elusive addictions. Indeed, persistent fulfillment of Khaderian adaptive preferences look like cases of addictions. Specifically, the successful fulfillment of a Khaderian adaptive preference undermines one's goals for flourishing.

Under this argument, whether a person's stated preferences undermine her true goals is not a matter of how autonomously she develops her preferences or how subjectively defensible they are. Rather, it is about how the content of the preference compares to how that person objectively flourishes. To display the need for such a normative, content-sensitive account of adaptive preference, let us go over two examples of an autonomous value-shift caused by an external domain. We will see that there are even autonomous value shifts that we would like to call inappropriately adaptive preferences, and potentially addictions.

We will imagine a value shift from substance-use. Suppose Marty is depressed. He, in fact, values very little. His misfortune has led to become ambivalent, if not bored or sad, most of the time. Now suppose he starts taking an antidepressant in order to have a richer life. Over time, Marty reports changes to his values. Suddenly, he pursues jobs he never pursued before, and drinks less alcohol. He enjoys reading and trying to find friends. When asked about this shift of values, his responses are not deferential. He doesn't say, "This is how I should feel because of the medication." Rather, he says things like, "I need to get back on my feet! I feel renewed and forward-looking." In other words, Marty has reasons, personal reasons, autonomously derived reasons, for why his values changed.

The Non-Autonomy model of adaptive preference would conclude that Marty's values are not adaptive preferences because Marty's reasons for his preferences are defensible, not deferential. Khader's (2011) deliberative perfectionist approach would also conclude that Marty's values are not adaptive preferences, but it would be because, even with a vague conception of human flourishing, it is arguably easy to see that valuing little and being depressed is less conducive to human flourishing than Marty's new values. Indeed, it turns out that, in this case, using either model of adaptive preference would lead us to conclude that Marty's

preferences are not adaptive. When used as a lens for the current model of addiction, we would say that he is not addicted to his antidepressants. Even if the substance induced these values along some causal chain, we think of Marty's new values as more acceptable, not as adaptive preferences, because either: 1) He autonomously defends his reasons for the shift, or 2) His new values are more conducive to human flourishing. I would like to show that, to assess 'proper directionality' of value-shifts, we must rely on the second justification listed above. That is, we must employ a normative, content-sensitive, account of well-being—Khader's model.

To show you this point, now imagine the reverse. Suppose Marty Prime has been a happy person his whole life. He has a rich career, warm family, fun social life, and lives in a gracious community of welcoming people. Suppose Marty Prime starts using methamphetamines for even further relaxation. Over time, he grows to despise all that which he used to love. He cuts ties to his family and friends. He quits his job. When asked about this change in values, he seriously defends his newfound preferences. Is Marty Prime addicted to meth?

First, we should notice how the Non-Autonomy model handles this account. Since Marty Prime's new values are, again, defended on non-deferential grounds, the Non-Autonomy model would suggest that his values are not adaptive preferences. We would say that these are Marty Prime's genuine values.

Under Khader's model, we need to first note that Marty Prime started out within our vague conception of human flourishing. So, right off the bat, Marty Prime's value-shift is not a Khaderian adaptive preference because he did not acclimate to a lack of anything. Since it is not an adaptive preference, we say (like we did with the Non-Autonomy model) that he seriously and genuinely grew to despise all that which he used to love. His new preferences may not be conducive to our minimal and vague sense of flourishing anymore, but he may not have

developed these preferences due to being deprived. There's not enough "adaptiveness" to this preference.

However, suppose Marty Prime's shift of values from friendly to misanthropic was indeed encouraged by his plummet into a state of deprivation. Even if they were not Khaderian adaptive preferences at first, they developed into Khaderian adaptive preferences over time. Indeed, when asked about his change of values, Marty defends them autonomously, but he admits that this is not what flourishing looks like. When one's immediate preferences are not conducive to a sense of human flourishing, and these preferences developed in response to and hinge on deprived circumstances, we are dealing with a Khaderian "inappropriately adaptive preference." We might **also** be dealing with an addiction, but not necessarily. Recall that the Non-Autonomy model of adaptive preference would still call these preferences authentic—even if they developed in deprived circumstances. All that was needed for autonomy in the Non-Autonomy model was defensible reasons for one's preferences (against at least hypothetical opponents). The deliberative perfectionist approach saves deprived individuals from being thought of as genuinely wanting what they say they want, even with "defensible"—which often caches out as 'contextually sensitive'—reasons.

I say "saves" under the impression that it is not within any vague sense of flourishing to want to, say, lose your friends, jobs, and home to alcohol use. With Khader's approach, the deprived circumstances in which one's adaptive preference develops shoulders some of the burden of having that preference. When used as a lens for the current model of addiction, it looks like this: Given that addiction is persistent dependence on that which undermines one's goals, the persistent successful achievement of an inappropriately adaptive preference would count as an

addiction to that preference. More specifically, it would be a persistent unsuccessful dependence on a preference for the fulfillment of one's flourishing.

An Amendment: Omitting Unachievable Hypothetical Counterfactuals

Treating someone's actions as instances of pathological use only seems to make sense if the individual had available external options. Sometimes, given Khader's deliberative perfectionist approach, people's adaptive preferences will be discordant with a practically *achievable* sense of flourishing, and sometimes it will be discordant with a practically *unachievable* sense of flourishing. When it is the former, it seems it could count as an addiction under the current model. When it is the latter, it clearly does not count as an addiction under the current model; perhaps instead the issue is environmental or circumstantial, and not within the patient at all.

4. The Absurd Commitment

I want to remind the reader that Khader did not suggest that her deliberative perfectionist approach to adaptive preference can be used as a lens through which we can identify addictions. The reason that my thesis is important is because Khader's approach to adaptive preference is what I see to be the best possible argument for a case of an elusive addiction—whereby an unsuccessful dependence camouflages itself as a successful dependence. However, this argument fails.

Here is where Khader's approach reaches absurdity. We have concluded that the satisfaction of a Khaderian adaptive preference, whose flourishing alternative is achievable, counts as an addiction. However, this implies that one's actions should be guided by things that

are not only counterfactual to one's situation, but also retrospectively counterfactual. One way I can demonstrate the absurdity of this commitment is to ask you to imagine the following conversation between Marty and his therapist.

THERAPIST: Marty. You should drink less for the sake of your health.

MARTY: I prefer to drink this much though.

THERAPIST: But imagine your flourishing. Do you think that a flourishing Marty would drink this much?

MARTY: No. If I were flourishing, I would drink less.

THERAPIST: Okay, that is what I mean. If you want to flourish, you'll need to drink less.

MARTY: Why?

THERAPIST: Because if you were flourishing, you would want to.

MARTY: But I am not flourishing, so I do not want to.

THERAPIST: By undermining your concept of flourishing, you are unsuccessfully dependent on alcohol for the achievement of flourishing. You seem addicted to alcohol, Marty.

I would like to defend Marty here. There are at least two reasons that it is absurd to conclude that Marty's, or anyone's, actual preferences should be guided by one's counterfactual flourishing preferences, even if the object of those preferences are achievable. Firstly, achievement of counterfactual flourishing is not representative of one's ability to flourish in their actual circumstances, and one's actual psychological abilities are what we are concerned with in psychiatric assessments. Terlazzo (2018) explains this well, writing,

If everyone agrees that avoiding frustration has prudential value, then our inquiry should focus not on what persons should do given limited options, but on what would be prudentially best for persons if we could make other external options available. That is, we should be interested in what is prudentially best given facts about persons, not facts about external obstacles. (Terlazzo 2018)

The fact about Marty is that he does not want to be sober given his development in deprived circumstances. Simply asking him what flourishing looks like, imagining nonexistent external options, does not erase the reality of his situation.

5. Conclusion

If I am right, and the current model is correct, then another important takeaway is that it may become easier to plan out how to help some people whose diagnosed substance use disorders register comorbidly with other ailments. Dependence is a tertiary relation between the dependent, the depended, and the reason or goal to be achieved. Alcohol, or any substance, use is not devoid of a goal, and remembering that will lead us to cases where remedial attention to other ailments directly remediates the desire to use the problematic substance. Indeed, one's substance use, especially when comorbid with other ailments, may understandably be an attempt at self-medication (Pickard 2018).

The deliberative perfectionist approach shows us that Marty's preference for alcohol is not fully representative of his most freely selected preferences. Indeed, he feels forced into this preference. That is helpful for Marty's *moral* standing. Meaning: We might say that we understand Marty's preference for alcohol better, given that it developed in response to deprived

circumstances. That is, we might blame him less morally. However, addiction diagnoses are not about moral failures. They are about prudential failures. Satisfaction of a Khaderian adaptive preference is not a case of addiction because nothing has ‘gone wrong’ prudentially. Something seems to have gone wrong environmentally. Blame deprivation for the acquisition of deleterious preferences. However, do not blame the agent’s psychological processes, because Khader’s inappropriately adaptive preferences do not develop via inappropriate uses of the mind.

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